

Rosewood General Hospital

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COMPREHENSIVE PATIENT MEDICAL REPORT

Confidential — For Authorized Medical Personnel Only

1. PATIENT INFORMATION

Patient Name	Mr. Arjun Mehta
Date of Birth	14 March 1981 (Age: 43 years)
Gender	Male
Blood Group	B+ (B Positive)
Patient ID	RGH-2024-08741
Aadhar / ID	XXXX-XXXX-7823
Contact Number	+91-98450-23671
Emergency Contact	Mrs. Priya Mehta (Spouse) — +91-98450-12345
Address	42, 3rd Cross, Koramangala 5th Block, Bengaluru — 560095
Insurance Provider	Star Health Insurance Policy No: SHI-BLR-441928
Referring Physician	Dr. Sunita Rao, MD — Koramangala Family Clinic
Admission Date	12 November 2024
Discharge Date	17 November 2024
Ward / Room	General Medicine Ward — Room 214B

2. CHIEF COMPLAINTS

- Persistent headache and dizziness for the past 3 weeks
- Intermittent chest tightness, especially during physical exertion
- Shortness of breath on climbing stairs (2 flights)
- Fatigue and weakness — unable to complete routine daily activities
- Blurred vision episodes lasting 5–10 minutes, occurring 2–3 times/week
- Nocturia (urination at night) — approximately 3 times per night
- Mild bilateral ankle swelling noted for 10 days

3. PAST MEDICAL HISTORY

Hypertension	Diagnosed 2018 — currently on antihypertensive therapy
Type 2 Diabetes Mellitus	Diagnosed 2020 — managed with oral hypoglycaemics
Dyslipidaemia	Diagnosed 2021 — on statin therapy
Allergies	Penicillin (rash), Sulfonamides (urticaria) — documented
Surgical History	Appendectomy — 2009 (City General Hospital, Mumbai)
Family History	Father: Hypertension, IHD (deceased age 67) Mother: Type 2 Diabetes Sibling: Hypertension
Social History	Non-smoker (quit 2015, 10 pack-year history) Occasional alcohol (< 2 units/week) Sedentary desk job — software engineer Low physical activity level

4. VITAL SIGNS ON ADMISSION

Parameter	Value	Normal Range	Status
Blood Pressure	158 / 96 mmHg	< 120/80 mmHg	HIGH ▲
Heart Rate	88 bpm	60–100 bpm	Normal ✓
Respiratory Rate	18 breaths/min	12–20/min	Normal ✓
Temperature	37.1 °C	36.1–37.2 °C	Normal ✓
SpO2 (Oxygen Sat.)	96%	> 95%	Normal ✓
Weight	87 kg	—	—
Height	172 cm	—	—
BMI	29.4 kg/m ²	18.5–24.9	Overweight ▲
Waist Circumference	98 cm	< 90 cm (male)	HIGH ▲

5. LABORATORY INVESTIGATIONS

5A. Complete Blood Count (CBC) — 12 Nov 2024

Test	Result	Unit	Reference Range	Flag
Haemoglobin	12.4	g/dL	13.5–17.5	LOW ▼
RBC Count	4.2	mil/μL	4.5–5.9	LOW ▼
WBC Count	8,200	cells/μL	4,000–11,000	Normal
Platelet Count	2,45,000	/μL	1,50,000–4,00,000	Normal
Haematocrit (PCV)	38.5	%	40–54	LOW ▼
MCV	80	fL	80–100	Normal

MCH	26	pg	27–33	LOW ▼
Neutrophils	62	%	50–70	Normal
Lymphocytes	28	%	20–40	Normal
ESR	34	mm/hr	0–20	HIGH ▲

5B. Metabolic Panel & Lipid Profile — 12 Nov 2024

Test	Result	Unit	Reference	Flag
Fasting Blood Glucose	162	mg/dL	70–99	HIGH ▲
HbA1c	8.2	%	< 5.7 (normal) < 7.0 (diabetic target)	HIGH ▲
Serum Creatinine	1.3	mg/dL	0.7–1.2	HIGH ▲
eGFR	62	mL/min	> 60	Borderline
Blood Urea Nitrogen	22	mg/dL	7–20	HIGH ▲
Serum Sodium (Na+)	139	mEq/L	135–145	Normal
Serum Potassium (K+)	3.6	mEq/L	3.5–5.0	Normal
Total Cholesterol	218	mg/dL	< 200	HIGH ▲
LDL Cholesterol	142	mg/dL	< 100	HIGH ▲
HDL Cholesterol	38	mg/dL	> 40 (male)	LOW ▼
Triglycerides	198	mg/dL	< 150	HIGH ▲
VLDL	39.6	mg/dL	< 30	HIGH ▲
Serum Uric Acid	7.2	mg/dL	3.5–7.2	Borderline
ALT (SGPT)	42	U/L	7–56	Normal
AST (SGOT)	38	U/L	10–40	Borderline
Alkaline Phosphatase	88	U/L	44–147	Normal
Total Bilirubin	0.9	mg/dL	0.2–1.2	Normal
Serum Albumin	3.8	g/dL	3.5–5.0	Normal
TSH	2.4	mIU/L	0.4–4.0	Normal
Urine Microalbumin	82	mg/g Cr	< 30	HIGH ▲

Note: Elevated microalbuminuria suggests early diabetic nephropathy. Repeat testing in 3 months recommended. Lipid profile indicates mixed dyslipidaemia. Mild normocytic anaemia noted — further iron studies and B12/folate ordered.

6. INVESTIGATIONS — ECG & IMAGING

12-Lead ECG Report — 12 Nov 2024, 09:15 AM

Rhythm	Normal Sinus Rhythm
Heart Rate	88 bpm
PR Interval	164 ms (normal)
QRS Duration	92 ms (normal)
QTc Interval	438 ms (normal)
ST Segment	No ST elevation or depression
T Waves	Flattening in leads V4–V6 (possible ischaemic changes)
LVH Criteria	Sokolow-Lyon index: 38 mm — meets LVH criteria
Impression	Left Ventricular Hypertrophy with non-specific ST-T changes. Clinical correlation with 2D Echo recommended.

2D Echocardiogram — 13 Nov 2024

Left Ventricular EF	52% (lower limit of normal — mildly reduced)
LV Wall Motion	Mild hypokinesia of inferior wall segments
LV Wall Thickness	Interventricular Septum: 12 mm (thickened) Posterior Wall: 11 mm
LV Internal Dimensions	LVEDD: 52 mm LVESD: 36 mm
Left Atrium	Mildly dilated — 42 mm
Right Heart	Normal RV size and function. RA normal.
Valves	Mild mitral regurgitation (Grade I) Aortic valve — tricuspid, no stenosis
Pericardium	No pericardial effusion
Impression	Concentric LV hypertrophy with mildly reduced EF (52%). Mild MR. Findings consistent with hypertensive heart disease. Cardiology follow-up recommended.

Chest X-Ray PA View — 12 Nov 2024

Cardiac Shadow	Mildly enlarged — CTR = 0.54 (normal < 0.50)
Lung Fields	Clear. No consolidation, effusion, or pneumothorax
Costophrenic Angles	Acute bilaterally
Mediastinum	Central, not widened
Bones & Soft Tissue	No bony abnormality detected

Impression	Mild cardiomegaly. No acute pulmonary pathology.
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7. FINAL DIAGNOSIS

1. Primary	Uncontrolled Hypertension (Grade 2) with Hypertensive Heart Disease
2. Secondary	Type 2 Diabetes Mellitus — Poorly Controlled (HbA1c 8.2%)
3. Complication	Early Diabetic Nephropathy (Microalbuminuria — 82 mg/g Cr)
4. Cardiac	Concentric Left Ventricular Hypertrophy with mildly reduced EF (52%)
5. Haematological	Mild Normocytic Anaemia — likely mixed aetiology (Iron deficiency vs. Chronic disease)
6. Metabolic	Mixed Dyslipidaemia — Elevated LDL, Low HDL, Elevated Triglycerides
7. Anthropometric	Overweight (BMI 29.4) with Central Obesity

8. TREATMENT & MEDICATION PLAN

Antihypertensive Therapy:

#	Medication	Dose	Frequency	Route	Duration
1	Amlodipine	10 mg	Once daily (AM)	Oral	Ongoing
2	Telmisartan	80 mg	Once daily (AM)	Oral	Ongoing
3	Indapamide SR	1.5 mg	Once daily (AM)	Oral	Ongoing
4	Metformin SR	1000 mg	Twice daily	Oral	Ongoing
5	Glimepiride	2 mg	Once daily (pre-breakfast)	Oral	Ongoing
6	Rosuvastatin	20 mg	Once daily (PM)	Oral	Ongoing
7	Fenofibrate	145 mg	Once daily (PM)	Oral	Ongoing
8	Aspirin (Enteric Coated)	75 mg	Once daily (PM)	Oral	Ongoing
9	Iron (Ferrous Sulphate)	200 mg	Twice daily	Oral	3 months
10	Folic Acid	5 mg	Once daily	Oral	3 months
11	Vitamin B12	1500 mcg	Once daily	Oral	3 months

■■ **ALLERGY ALERT:** Patient is allergic to Penicillin and Sulfonamides. Do NOT prescribe any Sulpha-based diuretics (e.g., Hydrochlorothiazide). Indapamide chosen as it is a non-sulphonamide diuretic.

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9. LIFESTYLE & DIETARY RECOMMENDATIONS

Diet:

DASH Diet strictly recommended. Reduce sodium intake to < 1500 mg/day. Avoid pickles, papads, processed meats, canned foods, and restaurant/fast food. Increase potassium-rich foods: banana, sweet potato, spinach, lentils. Limit saturated fats — avoid ghee, butter, full-fat dairy, red meat. Target carbohydrate intake: 45–60g per meal (glycaemic control). Increase dietary fibre to 30–35g/day.

Physical Activity:

Target 150 minutes moderate aerobic exercise per week (30 min x 5 days). Start with 20-minute brisk walks, gradually increase. Avoid strenuous exercise until cardiac clearance obtained. Resistance training 2x/week after 4 weeks — with physiotherapist guidance.

Weight Management:

Target weight loss: 5–7 kg over 6 months (approximately 0.5–1 kg/week). Target BMI: < 25.0. Waist circumference target: < 90 cm.

Alcohol & Smoking:

Complete alcohol cessation strongly recommended (worsens hypertension and triglycerides). Patient is a non-smoker — reinforce avoidance of passive smoking.

Sleep & Stress:

Target 7–8 hours sleep per night. Screen for Obstructive Sleep Apnoea (OSA) — snoring history noted. Stress management: yoga, meditation recommended. Consider psychology referral if work stress continues.

Blood Pressure Monitoring:

Home BP monitoring twice daily (morning and evening). Target BP: < 130/80 mmHg. Keep a BP diary — bring to every follow-up visit. Recommended device: validated upper-arm BP monitor.

Blood Glucose Monitoring:

Fasting blood glucose daily (pre-breakfast). Post-meal glucose check (2 hours after meals) 3x/week. HbA1c target: < 7.0% in 3 months.

10. FOLLOW-UP & REFERRAL PLAN

2 Weeks	Review blood pressure control. Check compliance with medications. Review home BP diary. Repeat serum electrolytes (K+ monitoring on diuretic).
6 Weeks	Repeat fasting glucose and HbA1c. Review anaemia — repeat CBC + iron studies. Nephrology referral if microalbuminuria persists.
3 Months	Cardiology review with repeat 2D Echo. Ophthalmology — Diabetic retinopathy screening (overdue). Full metabolic panel repeat. Reassess weight and BMI.
6 Months	HbA1c repeat. Annual diabetic foot examination. Urine microalbumin repeat. Medication review.
Referrals Initiated	1. Cardiology — Dr. Rakesh Sharma (Appointment: 02 Dec 2024) 2. Nephrology — Dr. Anita Bose (Appointment: 15 Dec 2024) 3. Ophthalmology — Dr. Meera Pillai (Appointment: 20 Dec 2024) 4. Dietitian — Ms. Kavitha Nair (Appointment: 22 Nov 2024) 5. Physiotherapist — Mr. Suresh Rao (Appointment: 25 Nov 2024)

Emergency Instructions	Visit Emergency immediately if: • BP > 180/110 mmHg despite medications • Severe chest pain or pressure • Sudden weakness, facial drooping, speech difficulty (stroke symptoms) • Blood glucose < 70 mg/dL with symptoms (hypoglycaemia) • Severe shortness of breath at rest Emergency: 108 (Ambulance) Hospital: +91-80-4567-8911
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11. PHYSICIAN SIGN-OFF

Attending Physician	Consulting Cardiologist	Department Head
Dr. Priya Venkataraman, MD	Dr. Rakesh Sharma, DM (Cardio)	Dr. Suhas Kulkarni, MD
General Medicine	Cardiology	Internal Medicine
Reg. No: KMC-28741	Reg. No: KMC-19234	Reg. No: KMC-11092
Date: 17 Nov 2024	Date: 13 Nov 2024	Date: 17 Nov 2024
Signature: _____	Signature: _____	Signature: _____

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